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NATURE OF PROCEEDINGS: Hearing on Motion to Quash Service
of Summons; Demurrers

The Motion to Quash Service of Summons is denied.

The Demurrer to the Fourth Cause of Action filed by Children's Hospital
Los Angeles Medical Group, Inc. is overruled.

The Demurrer filed by Lara Nelson, M.D. is overruled.

BACKGROUND

Plaintiffs Andrea Steinhoff and Michael DiGiannantonio, individually and as successors in interest to decedent Miles DiGiannantonio (Plaintiffs), filed this action against Children's Hospital of Los Angeles CHLA, four physicians who treated their son, Children's Hospital of Los Angeles Medical Group, Inc. (Medical Group), and Lara Nelson, M.D. (Dr. Nelson), alleging that their minor son died of an intracranial hemorrhage after he was cannulated to venoarterial extracorporeal membrane oxygenation (VA ECMO) instead of the venovenous modality (VV ECMO) his condition indicated.

On February 9, 2026, the Court granted Children's Hospital of Los Angeles's Application for Determination of Good Faith Settlement and dismissed it with prejudice.

Plaintiffs filed the original Complaint on March 15, 2023. They filed the First Amended Complaint on May 13, 2026, filed an amendment substituting Dr. Nelson for Doe 2 on May 22, 2026, and filed the Second Amended Complaint (SAC)

by stipulation on June 11, 2026.

The causes of action in the (SAC) are: (1) Professional Negligence; (2) Negligence, withdrawn; (3) Wrongful Death; and (4) Fraud by Concealment.

Dr. Nelson filed a Motion to Quash Service of Summons and a Demurrer. Medical Group also filed a Demurrer. Plaintiffs filed oppositions to all the motions, which are all discussed below.

LEGAL STANDARD- MOTION TO QUASH

A defendant may serve and file a notice of motion “[t]o quash service of summons on the ground of lack of jurisdiction of the court over him or her.” (Code Civ. Proc., § 418.10, subd. (a)(1).) A defendant may make that motion and simultaneously demur. No act by a party who makes the motion, including filing a demurrer, constitutes an appearance unless the court denies the motion. (Code Civ. Proc., § 418.10, subd. (e)(1).) When a defendant challenges jurisdiction by moving to quash, “the burden is on the plaintiff to prove the existence of jurisdiction by proving, inter alia, the facts requisite to an effective service.” (Dill v. Berquist Construction Co. (1994) 24 Cal.App.4th 1426, 1440.) Where a plaintiff serves a defendant under a Doe amendment that Code of Civil Procedure section 474 did not authorize, the service is improper and is quashed. (Optical Surplus, Inc. v. Superior Court (1991) 228 Cal.App.3d 776, 783-784 (Optical Surplus, Inc.).)

Code of Civil Procedure section 474 provides in relevant part, “When the plaintiff is ignorant of the name of a defendant, he must state that fact in the complaint, or the affidavit if the action is commenced by affidavit, and such defendant may be designated in any pleading or proceeding by any name, and when his true name is discovered, the pleading or proceeding must be amended accordingly”

Code of Civil Procedure section 474 does not confine the required ignorance to ignorance of a name. “[E]ven though the plaintiff knows of the existence of the defendant sued by a fictitious name, and even though the plaintiff knows the defendant’s actual identity (that is, his name), the plaintiff is ‘ignorant’ within the meaning of the statute if he lacks knowledge of that person’s connection with the case or with his injuries.” (General Motors Corp. v. Superior Court (1996) 48 Cal.App.4th 580, 593-594 (General Motors).) The statute asks about facts and not about the plaintiff’s legal characterization

of them. “Ignorance of the facts giving rise to a cause of action is the ‘ignorance’ required by section 474, and the pivotal question is, “did plaintiff know facts?” not “did plaintiff know or believe that she had a cause of action based on those facts?”” (Id. at p. 594.)

“[T]he relevant inquiry when the plaintiff seeks to substitute a real defendant for one sued fictitiously is what facts the plaintiff actually knew at the time the original complaint was filed.” (Fuller v. Tucker (2000) 84 Cal.App.4th 1163, 1170.) “The fact that the plaintiff had the means to obtain knowledge is irrelevant.” (General Motors, supra, 48 Cal.App.4th at p. 594.) The statute “does not impose upon the plaintiff a duty to go in search of facts she does not actually have at the time she files her original pleading.” (Id. at p. 596.) A plaintiff who is ignorant of the requisite facts has no duty to exercise reasonable diligence to obtain facts she should have known. “Section 474 imposes no such duty.” (Hahn v. New York Air Brake LLC (2022) 77 Cal.App.5th 895, 900 (Hahn).)

A plaintiff “does not relinquish her rights under section 474 simply because she has a suspicion of wrongdoing arising from one or more facts she does know.” (General Motors, supra, 48 Cal.App.4th at p. 595.) Code of Civil Procedure section 474 “allows a plaintiff in good faith to delay suing particular persons as named defendants until he has knowledge of sufficient facts to cause a reasonable person to believe liability is probable.” (McOwen v. Grossman (2007) 153 Cal.App.4th 937, 944 (McOwen).)

“[T]he omission of the defendant’s identity in the original complaint must be real and not merely a subterfuge for avoiding the requirements of section 474.” (Woo v. Superior Court (1999) 75 Cal.App.4th 169, 177 (Woo).)

Likewise, “[i]gnorance of the facts giving rise to a cause of action, like ignorance of the true name, should be real and not feigned. [Citations]” (Optical Surplus, Inc., supra, 228 Cal.App.3d at p. 784.)

ANALYSIS- MOTION TO QUASH

Dr. Nelson challenges service on one ground, that Plaintiffs served her under a Doe amendment that Code of Civil Procedure section 474 did not permit.

I. The Required Ignorance

Dr. Nelson contends that Plaintiffs were ignorant of nothing, because Plaintiffs alleged in the original Complaint that CHLA lacked an appropriately sized VV cannula, that VA ECMO was recommended, that the parents were not advised of its greater hemorrhage risk, and that Miles died of an intracranial hemorrhage while on VA ECMO. (Reply, at p. 3:18-27.)

Under Code of Civil Procedure section 474, the required ignorance runs to a person's connection with the injury and not to the existence of a claim. A plaintiff may know a defendant's name and know that she took part in the events, and still be ignorant under the statute where the plaintiff "lacks knowledge of that person's connection with the case or with his injuries." (General Motors, *supra*, 48 Cal.App.4th at pp. 593-594.)

Here, Plaintiffs pleaded the ECMO theory in the original Complaint and charged it against a physician they did not name. First, Plaintiffs alleged that CHLA did not have the proper sized VV cannula in stock, that Dr. Toomey told the parents VA ECMO was an acceptable alternative, and that she never advised the parents of the greater risk of hemorrhage on VA ECMO. (Compl., ¶ 18.) Second, they charged the recommendation and the nondisclosure against "Dr. Toomey and DOES 1 through 200." (Compl., ¶ 32.) Third, they alleged that the Doe defendants' true names were unknown to them and that each was responsible in some manner for the events described in the Complaint. (Compl., ¶¶ 10-11.) Thus, the original Complaint states a cause of action against the physician who bore the disclosure duty and leaves that physician unnamed. Code of Civil Procedure section 474 authorizes that designation.

Under Dr. Nelson's reading, ignorance would end the moment a plaintiff pleads the claim. Her contention asks the second of the two questions General Motors distinguishes, whether the plaintiff knew she had a cause of action, and not the first, whether she knew the facts. (See General Motors, *supra*, 48 Cal.App.4th at p. 594.) Dr. Nelson also relies on the rule that a plaintiff "need not be aware of each and every detail concerning a person's involvement before the plaintiff loses his ignorance." (*Ibid.*)

That clause is the concession in a sentence that holds a plaintiff "does not relinquish her rights under section 474 simply because she has a suspicion of wrongdoing arising from one or more facts she does know." (*Id.* at p. 595.) The rule governs a plaintiff who knows a person's connection with the injury and lacks detail about it. Plaintiffs lacked knowledge of the connection

itself, and not a detail of a connection they already knew.

II. Actual Knowledge

Dr. Nelson contends that the CHLA chart gave Plaintiffs the facts that they needed, because Plaintiffs admit that they received the complete record in April 2022 and the chart identifies her by name as an attending physician in the intensive care unit during the admission. (Mot., at p. 4:19-28.)

Dr. Nelson states the inquiry as what Plaintiffs “actually knew (or reasonably should have known)” when suit was filed, and attributes that formulation to Fuller. (Mot., at p. 4:11-13.) Fuller contains no such parenthetical. Moreover, the court in Hahn rejected the reasonable-diligence standard that the parenthetical imports. (Hahn, *supra*, 77 Cal.App.5th at p. 900.) The question is what Plaintiffs knew on March 15, 2023.

Here, Plaintiffs did not then know facts that would have caused a reasonable person to believe Dr. Nelson's liability probable. First, both parents declare that they did not know that Dr. Nelson was the ECMO consultant responsible for the modality decision and the risk discussion, that she had directed a search for a properly sized VV cannula, or that she knew one had been located at Loma Linda. (Steinhoff Decl., ¶ 4; DiGiannantonio Decl., ¶ 4.) Second, counsel declares that she reviewed the CHLA records before filing and found nothing indicating that Dr. Nelson bore that responsibility. (Teixeira Decl., ¶¶ 5, 9.) Third, the records name other physicians in the consent process. The consent form identifies Lauren Baumann, M.D. as the practitioner who explained the procedure, the risks, and the alternatives, and the operative report identifies Eugene Kim, M.D. as surgeon and Dr. Baumann as assistant. (Teixeira Decl., ¶¶ 7-8, Exs. 2-3.) Finally, the note that Dr. Nelson signed after cannulation records that the decision “was discussed with the family by both the surgical and PICU teams.” The note neither names the physician who conducted that discussion nor identifies her as the ECMO consultant. (Teixeira Decl., ¶ 6, Ex. 1.)

The chart page that Dr. Nelson submitted is a progress note dated December 9, 2021, a week before ECMO was raised. The note identifies her as “Nelson (Attending) MD, Lara P Attending - Pediatric Intensive Care.” (Mahmoudi Decl., ¶ 5, Ex. 3.) Dr. Nelson replies that the title places her within the PICU team to which her later note credits the family discussion. (Reply, at p. 6:2-4.)

However, the note that Dr. Nelson signed attributes the discussion to two teams and names no physician who conducted it. (Teixeira Decl., ¶ 6, Ex. 1.) Nothing in the pages before the Court identifies Dr. Nelson as a member of either team on December 16 or December 17, 2021. The chart identifies Dr. Toomey as the PICU attending on December 16. Dr. Toomey testified that she contacted whichever attending provider was the ECMO consultant for the day, and that she did not know who the consultant was for Miles. (Toomey Depo., at p. 59:7-24.) Plaintiffs knew from the chart that Dr. Nelson attended part of a 25-day admission. That fact would not cause a reasonable person to believe her liability probable. (McOwen, *supra*, 153 Cal.App.4th at p. 944.)

Dr. Nelson's reliance on *Dover*

v. Sadowinski (1983) 147 Cal.App.3d 113 is misplaced. There, the defendant was plaintiff's wife's attending physician, had met with and "informed plaintiff that he was the doctor in charge of plaintiff's wife's care." (Id. at pp. 117, 118.) Counsel conceded at the hearing, "We knew he was involved, but we had no idea, Your Honor, how deeply as a negligent individual, he was involved." (Id. at p. 118.) The court concluded that the claimed lack of knowledge was feigned. (Ibid.) Here, both parents declare that Dr. Nelson did not participate in the ECMO consent discussions with them and never identified herself as the physician responsible for the modality decision. (Steinhoff Decl., ¶ 5; DiGiannantonio Decl., ¶ 5.)

III. Notices of Intent

Dr. Nelson treats the notice of intent that she received as an admission that Plaintiffs had identified her by name and asserted a professional negligence claim against her three months before they filed suit. (Reply, at p. 4:10-17.)

Andrea Steinhoff and Michael DiGiannantonio sent the notices on December 23, 2022, before they retained counsel. (Mahmoudi Decl., ¶ 4, Ex. 2; Steinhoff Decl., ¶ 2; DiGiannantonio Decl., ¶ 2; Teixeira Decl., ¶ 3.) They sent the same letter to approximately 21 providers whose names appeared in the chart, including every attending physician for the hospitalization. (Steinhoff Decl., ¶¶ 2-3; DiGiannantonio Decl., ¶¶ 2-3; Teixeira Decl., ¶ 3, Ex. 5.) Their letter to Dr. Nelson states that "[d]ue to your professional negligence, and the professional negligence of others, Miles' condition worsened dramatically during the course of his 25-day hospital admission." The letter closes, "Your

acts and omissions were the proximate cause of Miles' wrongful death." (Mahmoudi Decl., ¶ 4, Ex. 2.) Between those sentences the letter lists 12 failures in the hospitalization, among them that Miles "was cannulated for VA-ECMO, which his condition did not warrant, instead of VV-ECMO, because CHLA did not have the proper-sized cannula," that "there was no attempt made to locate the correct cannula for VV-ECMO," and that "we were not apprised of the different risks between the two." (Ibid.)

Here, the notices show that Plaintiffs asserted a nondisclosure claim against every attending physician whose name they could read. The notices do not show that Plaintiffs knew which physician owed the disclosure.

The parents had a list of the things that had gone wrong during the hospitalization and the names of the providers who appeared in the chart. They had no document connecting Dr. Nelson to either of the two decisions that the letter complains of, the choice of modality and the disclosure of comparative risks. The letter asserts that no one searched for a VV cannula and that no one explained the difference in risk. The letter identifies no physician as the one who owed that explanation. Nor does the letter state that a properly sized cannula had been located. Both parents declare that they used the same general notice for every provider because they did not know which of the many providers involved in the hospitalization might be legally responsible. (Steinhoff Decl., ¶ 3; DiGiannantonio Decl., ¶ 3.)

Thus, the notices show that the parents suspected the providers whose names they could read. Plaintiffs did not relinquish their rights under Code of Civil Procedure section 474 on the strength of that suspicion. (See *General Motors*, supra, 48 Cal.App.4th at p. 595.)

In *Optical Surplus*, counsel wrote the later substituted defendant four months before suit. The letter charged that the defendant had "been actively engaged in the sale and solicitation of orders for the sale of eyeglass frames manufactured by Metzler [] in the state of California," in violation of the plaintiff's exclusive distribution rights. (*Optical Surplus, Inc.*, supra, 228 Cal.App.3d at p. 781.) There, the plaintiff knew both the defendant and the conduct complained of. Here, the letter charges Dr. Nelson with no specific conduct of her own and attributes none of the listed failures to a particular physician.

IV. Good Faith

Dr. Nelson contends that Plaintiffs feigned their ignorance, because they

served her with a statutory malpractice notice and then omitted her from the original Complaint. (Reply, at p. 8:7-10.)

Dr. Nelson reads Woo more broadly than the opinion permits.

There, the court addressed a plaintiff who had known the defendant's identity and forgotten it by the time of filing. The court held that a plaintiff in that position "must have at least reviewed readily available information likely to refresh his or her memory." (Woo, supra, 75 Cal.App.4th at p. 180.) The court applied that requirement to a plaintiff who "did not avail herself of readily available information that would have refreshed her recollection." Notably, the court added that the requirement "is not inconsistent with the cases that impose no duty of inquiry on plaintiffs who never knew the defendant's identity." (Ibid.) Plaintiffs do not claim a lapse of memory.

Here, Plaintiffs omitted Dr. Nelson's name from the original Complaint in good faith. First, Plaintiffs pleaded 200 Doe defendants under Code of Civil Procedure section 474 and alleged that each was responsible in some manner for the events described in the Complaint. (Compl., ¶¶ 10-11.) Second, they named the physicians that the chart connected to the acts they complained of. They charged the ECMO recommendation and the nondisclosure against "Dr. Toomey and DOES 1 through 200." (Compl., ¶ 32.) Third, they served a deposition notice on the Medical Group for its person most qualified concerning the search for and location of an appropriately sized VV cannula and the decision to proceed with VA ECMO. They deposed the PICU attending, and they deposed Dr. Nelson when the Medical Group produced her as the physician knowledgeable about the ECMO decision. (Teixeira Decl., ¶¶ 11-14, Exs. 6-8.) Thus, Plaintiffs did not omit Dr. Nelson's name as a subterfuge for avoiding the requirements of Code of Civil Procedure section 474. (Woo, supra, 75 Cal.App.4th at p. 177.)

Dr. Nelson also points out that the First Amended Complaint describes her conduct in detail while continuing to designate unknown Doe defendants. (Mot., at p. 5:15-21.) Plaintiffs filed that pleading in May 2026, three months after her deposition and only 9 days before naming her as a defendant. Plaintiffs could plead those facts in 2026 because their discovery succeeded. Their 2026 pleading does not show what they knew in March 2023.

The declarations of both parents and of counsel, and records that named other physicians in the ECMO modality decision and the consent discussion, support two findings. The Court finds that Plaintiffs did not know, on March

15, 2023, facts that would have caused a reasonable person to believe Dr. Nelson's liability probable. The Court further finds that Plaintiffs acted in good faith when they omitted her name from the original Complaint. Against that showing Dr. Nelson offers two documents, a progress note from a week before ECMO was raised and a letter sent in identical terms to approximately 21 providers. Neither establishes that Plaintiffs knew of her connection to the modality decision or to the disclosure. Plaintiffs have therefore carried their burden of proving the facts requisite to effective service.

Accordingly, Code of Civil Procedure section 474 authorized the amendment substituting Dr. Nelson for Doe 2, and the amendment relates back to the filing of the original Complaint. Service made under that amendment is proper. The Motion to Quash Service of Summons is denied.

LEGAL STANDARD - DEMURRER

When considering demurrers, courts read the allegations liberally and in context. (Wilson v. Transit Authority of City of Sacramento (1962) 199 Cal.App.2d 716, 720-21.) In a demurrer proceeding, the defects must be apparent on the face of the pleading or via proper judicial notice. (Donabedian v. Mercury Ins. Co. (2004) 116 Cal.App.4th 968, 994.) “A demurrer tests the pleading alone, and not the evidence or the facts alleged.” [Citation.]” (E-Fab, Inc. v. Accountants, Inc. Services (2007) 153 Cal.App.4th 1308, 1315.) As such, courts assume the truth of the complaint’s properly pleaded or implied factual allegations. (Ibid.) However, they do not accept as true deductions, contentions, or conclusions of law or fact. (Stonehouse Homes LLC v. City of Sierra Madre (2008) 167 Cal.App.4th 531, 538.)

ANALYSIS – DEMURRERS

Statute of Limitations (Dr. Nelson)

Code of Civil Procedure section 340.5 provides in relevant part, “In an action for injury or death against a health care provider based upon such person’s alleged professional negligence, the time for the commencement of action shall be three years after the date of injury or one year after the plaintiff discovers, or through the use of reasonable diligence should have discovered, the injury, whichever occurs first. In no event shall the time for commencement of legal action exceed three years unless tolled for any of the

following: (1) upon proof of fraud, (2) intentional concealment, or (3) the presence of a foreign body, which has no therapeutic or diagnostic purpose or effect, in the person of the injured person.”

Dr. Nelson contends that the First, Third, and Fourth Causes of Action are barred by the three-year outside limit in Code of Civil Procedure section 340.5, because Miles died on December 28, 2021 and Plaintiffs did not name her until 2026, and that the Doe substitution does not relate back because Plaintiffs were not genuinely ignorant of her identity.

A limitations defense reaches a pleading only on a narrow showing. “In order for the bar of the statute of limitations to be raised by demurrer, the defect must clearly and affirmatively appear on the face of the complaint; it is not enough that the complaint shows that the action may be barred.” (Marshall v. Gibson, Dunn & Crutcher (1995) 37 Cal.App.4th 1397, 1404-1405.)

Here, no limitations bar appears clearly and affirmatively on the face of the SAC. Plaintiffs allege that Miles died on December 28, 2021. (SAC, ¶ 45.) They also allege that Dr. Nelson was previously identified as Doe 2, and that the true names and capacities of the Doe defendants were unknown to them when they sued. (SAC, ¶¶ 5, 8.) Read together, those allegations plead an action commenced within three years against a fictitiously named defendant and a later substitution under Code of Civil Procedure section 474. Nothing in the SAC states what Plaintiffs knew about Dr. Nelson on March 15, 2023, which is the fact the defense depends on.

Dr. Nelson’s showing on that fact comes instead from a declaration and its exhibits, a section 364 notice, a page identifying her as an addressee of one, and a page of the hospital chart. (Mahmoudi Decl., ¶¶ 3-5, Exs. 1-3.) However, a demurrer tests the pleading and the matters subject to judicial notice, and it does not weigh evidence. (Code Civ. Proc., § 430.30, subd. (a); Blank v. Kirwan, supra, 39 Cal.3d at p. 318.)

Dr. Nelson’s remaining argument reads the SAC as alleging that she spoke with the parents about the ECMO modality on December 16, 2021. Plaintiffs allege the opposite. They allege that Dr. Nelson did not tell the parents a properly sized VV cannula was being sought, did not tell them one had been located at Loma Linda, did not explain that VV ECMO was indicated for isolated respiratory failure, and did not discuss the comparative risks of the two modalities. (SAC, ¶¶ 74-81.) They further allege that the parents never knew

Dr. Nelson was the physician responsible for those disclosures. (SAC, ¶ 72.)

Taking those allegations as true, the pleading alleges Plaintiffs' ignorance instead of negating it.

Additionally, the Court addresses the genuine ignorance issue in its order denying the Motion to Quash, and concludes that Plaintiffs have met their burden. Accordingly, the Demurrer to the First, Third, and Fourth Causes of Action on limitations grounds is overruled.

Fraud by Concealment (Dr. Nelson)

Dr. Nelson contends that the Fourth Cause of Action pleads no particularized facts of fraud, and that what Plaintiffs call concealment is a claim for lack of informed consent under another name.

Fraud is pleaded with particularity, and the pleading must show "how, when, where, to whom, and by what means the representations were tendered." (Lazar v. Superior Court (1996) 12 Cal.4th 631, 645.) The elements of fraudulent concealment are: "(1) the defendant must have concealed or suppressed a material fact, (2) the defendant must have been under a duty to disclose the fact to the plaintiff, (3) the defendant must have intentionally concealed or suppressed the fact with the intent to defraud the plaintiff, (4) the plaintiff must have been unaware of the fact and would not have acted as he did if he had known of the concealed or suppressed fact, and (5) as a result of the concealment or suppression of the fact, the plaintiff must have sustained damage." (Hahn v. Mirda (2007) 147 Cal.App.4th 740, 749 (Hahn).)

Here, Plaintiffs plead fraudulent concealment against Dr. Nelson with the particularity the rule demands. First, they name the person and her role, Dr. Nelson, the physician CHLA Medical Group assigned as the ECMO consultant for Miles's case and charged with deciding the modality and disclosing its risks, benefits, and alternatives. (SAC, ¶¶ 23, 72.) Second, a physician stands in a fiduciary relationship with the patient, and because Miles was a minor, the duty to disclose ran to the parents who decided for him. (SAC, ¶¶ 63-64; Hahn v. Mirda, supra, 147 Cal.App.4th at p. 748.) Third, they identify what was withheld: that VV ECMO was indicated for isolated respiratory failure, that a properly sized VV cannula was being sought and might be available nearby, that one had been located at Loma Linda, that waiting and transfer were available, and that VA ECMO carried a greater risk of stroke and neurologic injury and made hypertension harder to control in a child with normal cardiac output. (SAC, ¶¶ 74-83.) Fourth, they identify the time and the audience, the evening of

December 16, 2021, before cannulation, and Miles's parents. (SAC, ¶¶ 74-84.) Finally, they plead intent, reliance, causation, and harm, alleging that Dr. Nelson withheld those facts to obtain consent to VA ECMO, that the parents would have sought VV ECMO or transfer or delay had they known, and that Miles suffered the hemorrhage that killed him. (SAC, ¶¶ 84, 87-93.)

Citing Hahn Dr. Nelson states that fraud in this context requires conduct that "transcends ordinary negligence or a failure of informed consent", attributing it to Hahn at pages 748 and 749. (Dem., at p. 7; Reply, at p. 6.) No language to that effect appears in Hahn. On the contrary, the court noted, "Appellant pleaded a cause of action for fraud, not a cause of action for damages based on lack of informed consent. It is irrelevant that Ms. Hahn may not have been able to state a cause of action based on a theory of liability she did not allege." (Hahn, *supra*, 147 Cal.App.4th at p. 749.)

In Hahn, a patient pleaded professional negligence and fraudulent concealment against her treating physicians, alleging that they learned from a corrected re-read of her biopsy slides that she had never had the recurrent cancer for which she underwent chemotherapy and a radical mastectomy, agreed to tell her, and did not; her husband pleaded loss of consortium. (Hahn, *supra*, 147 Cal.App.4th at pp. 743-745.) The trial court sustained a demurrer without leave to amend, the patient dismissed her appeal, and the husband conceded that the physicians owed him no duty as her spouse, so the court examined whether the patient's causes of action were adequately pleaded. (Id. at pp. 745-746.) The court held that a complaint alleging each element of concealment against a treating physician states a claim, and that the availability or unavailability of an informed consent theory on the same facts does not change that result. (Id. at pp. 748-749; see also *Bigler-Engler v. Breg, Inc.* (2017) 7 Cal.App.5th 276, 321-323 [claims for breach of fiduciary duty and for intentional concealment against the same physician, each resting on the same nondisclosure, analyzed as professional negligence and as an intentional tort respectively].)

Here, the Fourth Cause of Action is not the First Cause of Action under another name. In the negligence count, Plaintiffs allege that the Medical Group's physicians negligently recommended VA ECMO and failed to inform the parents of the additional risks of that modality. (SAC, ¶ 52.) That count charges a failure to meet a standard of care. In the fraud count, Plaintiffs allege that Dr. Nelson knew a properly sized VV cannula had been located at a named

hospital, knew the comparative risks of the two modalities, and withheld both to obtain a consent she would not otherwise have had. (SAC, ¶¶ 76, 79, 82, 84, 90.) Whether a cannula had been found at Loma Linda is not a matter of professional judgment; Plaintiffs allege that Dr. Nelson knew it and did not say so. (SAC, ¶ 76.) A physician who discharges the disclosure duty carelessly has breached a standard of care. Plaintiffs allege instead that Dr. Nelson held material facts and withheld them so that the consent would be given. (SAC, ¶¶ 72, 76, 82.) Accordingly, the demurrer for failure to state a claim is overruled.

Uncertainty (Dr. Nelson)

Dr. Nelson also objects to the Fourth Cause of Action as uncertain. (Code Civ. Proc., § 430.10, subd. (f).) “A demurrer for uncertainty is strictly construed, even where a complaint is in some respects uncertain, because ambiguities can be clarified under modern discovery procedures.” (*Khoury v. Maly's of California, Inc.* (1993) 14 Cal.App.4th 612, 616.) Dr. Nelson identifies no ambiguity that prevents her from responding, and her memorandum takes up the concealment allegations one by one and answers them. (Dem., at pp. 6-7.) The demurrer for uncertainty is therefore overruled.

Corporate Particularity (Medical Group)

The Medical Group contends that the Fourth Cause of Action fails against it because Plaintiffs plead no facts identifying who at the corporation gave Dr. Nelson authority to conceal anything, and no deceptive intent by the entity itself.

Here, Plaintiffs plead each particular that the corporate rule requires. The Fourth Cause of Action incorporates the allegations that precede it. (SAC, ¶ 62.) The Court therefore reads the pleading as a whole.

First, Plaintiffs name the persons who withheld the information, Dr. Nelson and Dr. Vanessa Toomey. (SAC, ¶¶ 23, 63, 70.) Second, they allege that both physicians were Medical Group employees with authority to speak on its behalf, that both acted with actual and ostensible authority in obtaining informed consent and communicating treatment options, and that the Medical Group delegated responsibility for ECMO consent discussions and treatment recommendations to its employed physicians. (SAC, ¶¶ 65-67.) Third, they identify the audience, Miles's parents, who made the treatment decision for

their minor son. (SAC, ¶¶ 64, 72.) Fourth, they identify what was withheld and when. They allege that Dr. Nelson did not tell the parents that VV ECMO was indicated for isolated respiratory failure, that a properly sized VV cannula was being sought and had been located at Loma Linda, that waiting and transfer were available, or that VA ECMO carried greater neurologic and hypertension-related risks, all on December 16, 2021, before cannulation. (SAC, ¶¶ 74-83.) These allegations are sufficient to plead fraud by concealment.

Authority to Speak (Medical Group)

The Medical Group argues Plaintiffs must plead “who from [the Medical Group] gave Dr. Nelson the authority to commit the fraud.” (Dem., at p. 5:3-4.) The Medical Group adds that “[t]here must be action by the entity itself, as its own legal being.” (Dem., at p. 5:12-13.) The corporate rule requires authority to speak, not authority to commit the tort. (Lazar, *supra*, 12 Cal.4th at p. 645.) The Medical Group cites no authority requiring a plaintiff to plead that a corporate officer authorized the tort itself.

A principal is responsible for an agent’s wrongful acts committed in transacting the agency’s business. (Civ. Code, § 2338.) An employee’s “willful, malicious and even criminal torts may fall within the scope of his or her employment,” even where the employer did not authorize them. (Lisa M. v. Henry Mayo Newhall Memorial Hospital (1995) 12 Cal.4th 291, 296-297 (Lisa M.)) The Medical Group’s proposed rule would require Plaintiffs to plead an element the substantive law does not impose.

The Medical Group’s reliance on Tarmann v. State Farm Mut. Auto. Ins. Co. (1991) 2 Cal.App.4th 153 is misplaced. (Reply, at p. 4:14-18.) The plaintiff there could not name the individuals who made the promise she sued on. She alleged only that they were ““authorized agents of State Farm . . . cloaked with such authority”” and were adjusters or claims supervisors. (Id. at pp. 157-158.) Here, Plaintiffs name the physician, state the function that the Medical Group assigned her, and tie the concealment to her performance of that function.

Whether Dr. Nelson acted within the scope of her employment “presents a question of fact; it becomes a question of law, however, when ‘the facts are undisputed and no conflicting inferences are possible.’” (Lisa M., *supra*, 12 Cal.4th at p. 299.) Plaintiffs allege that the Medical Group assigned Dr. Nelson as the ECMO consultant, delegated the consent discussion to her, and that the concealment occurred while she

performed that delegated duty. (SAC, ¶¶ 38, 67, 72, 82.) The Court takes those allegations as true at this stage.

The Medical Group's Intent (Medical Group)

The Medical Group replies that authority to speak is necessary and not sufficient, and that the pleading must still allege a concealment by the corporation, including an intent to deceive attributable to the entity. (Reply, at pp. 2:24-3:6; see Dem., at p. 6:1-7.) The Medical Group adds that Plaintiffs attribute the intent collectively, to “Defendants” and to “CHLA Medical Group providers,” and allege no motive of the group and no communication from the group to these parents. (Reply, at p. 6:3-10.)

A corporation acts through its employees. California therefore imputes an employee's fraud to the employer on the same terms it imputes any other tort. In *Inter Mountain Mortgage*, a mortgage broker sued a real estate broker and the brokerage for the fraud of their employed loan representative, and the Court of Appeal reversed a summary judgment for the employers. (*Inter Mountain Mortgage, Inc. v. Sulimen* (2000) 78 Cal.App.4th 1434, 1437, 1442.) The court held that “[a]n employee's willful, malicious and even criminal torts may fall within the scope of his or her employment for purposes of respondeat superior, even though the employer has not authorized the employee to commit crimes or intentional torts.” (*Id.* at p. 1440.)

The Medical Group responds that these are vicarious liability decisions rather than fraud-pleading decisions. (Reply, at p. 4:3-10.) The Court does not cite them to displace the corporate pleading rule, which it applied above. They supply the element that the Medical Group says is missing. Where an employee acts within the authority her employer delegated to her, and in the course of that authority knows a material fact and withholds it to obtain a consent, the employer is charged with her intent.

Here, Plaintiffs plead that knowledge and that purpose, and they plead both as to a named physician rather than collectively. They allege that Dr. Nelson learned on December 16, 2021 that the correct VV cannula was available at Loma Linda and never told the family (SAC, ¶ 76); that she knew VA ECMO carried a higher risk of stroke and of neurologic injury and did not discuss the comparative risks (SAC, ¶ 79); and that she knew a child with normal cardiac output typically requires more blood pressure medication on VA ECMO and did not discuss that risk (SAC, ¶ 80). They further allege that she “knowingly

concealed VV availability, knowingly concealed comparative risks, knowingly concealed transfer options and knowingly concealed that a cannula had actually been located,” and that she and the other providers concealed those facts “in order to obtain consent for VA ECMO.” (SAC, ¶¶ 82, 84.)

The Medical Group's demand for a corporate motive asks for something fraudulent concealment does not require. The element is an intent to defraud, and Plaintiffs plead it. (Hahn, *supra*, 147 Cal.App.4th at p. 748.)

Fraud and Professional Negligence (Medical Group)

The Medical Group contends that the gravamen of the Fourth Cause of Action is medical negligence, and that the fraud count restates the negligence count with added adverbs. (Dem., at p. 6:12-19; Reply, at pp. 5:10-21.) In its Demurrer and Reply, Medical Group states the governing threshold as conduct that “transcends ordinary negligence or failure of informed consent,” first without quotes, and then with quotes, citing Hahn at pages 748 and 749. (Dem., at p. 6:15-17; Reply, at pp. 5:7-9, 5:19-21.) Neither that language, nor any similar language appears in that opinion. On the contrary the Hahn court noted “Appellant pleaded a cause of action for fraud, not a cause of action for damages based on lack of informed consent. It is irrelevant that Ms. Hahn may not have been able to state a cause of action based on a theory of liability she did not allege.” (Hahn, *supra*, 147 Cal.App.4th at p. 749.)

In Hahn, a patient and her husband sued her treating physicians. She pleaded professional negligence and fraudulent concealment, alleging that the physicians learned from a corrected re-read of her biopsy slides that she had never had the recurrent cancer for which she underwent chemotherapy and a radical mastectomy, agreed to tell her, and did not; her husband pleaded loss of consortium. The trial court sustained a demurrer without leave to amend. (Hahn, *supra*, 147 Cal.App.4th at p. 744.) The patient dismissed her appeal while it was pending, leaving her husband as the sole appellant, and he conceded that the physicians owed him no duty of care or disclosure as her spouse. (Id. at p. 745.) Because a loss of consortium claim “stands or falls based on whether the spouse of the party alleging loss of consortium has suffered an actionable tortious injury,” the court examined whether the patient's causes of action were adequately pleaded. (Id. at p. 746.) It concluded that they were, that her husband therefore had a viable loss of consortium claim, and it reversed the judgment on that claim. (Id. at pp. 747-749,

751-752.) The doctor-patient relationship is fiduciary, and a physician who conceals material medical information may be answerable in fraud as well as in negligence. (Id. at p. 748.)

The Medical Group distinguishes Hahn on the ground that the physicians there withheld a report establishing that the patient never had cancer. (Reply, at p. 5:5-7.) The distinction favors Plaintiffs. The physicians in Hahn withheld a discrete fact that the patient could not learn on her own, and Plaintiffs plead a similar concealment.

Here, Plaintiffs allege in the professional negligence count that the Medical Group's physicians negligently recommended VA ECMO and failed to inform the parents of the additional risks of that modality. (SAC, ¶ 52.) That count charges a failure to meet a standard of care. In the fraud count they allege that Dr. Nelson knew a properly sized VV cannula had been located at a named hospital, knew the comparative risks of the two modalities, and withheld both to obtain Plaintiffs' consent. (SAC, ¶¶ 76, 79, 82, 84, 90.) Whether a cannula had been found is not a matter of professional judgment. The fact was either disclosed to the parents or withheld from them and withholding it to induce a decision is not a want of care.

The Medical Group argues that the alleged concealment occurred while Dr. Nelson performed a consent function, and that the setting shows the claim to be the informed consent claim under another name. (Reply, at p. 4:23-27.) The setting of the alleged concealment does not determine the nature of the claim. A physician who conducts the consent discussion carelessly has breached a standard of care. A physician who withholds a fact she knows is relevant to that consent has done more than fall below a standard of care.

Accordingly, the Demurrer to the Fourth Cause of Action for failure to state a claim is overruled.

Uncertainty (Medical Group)

The Medical Group also objects to the Fourth Cause of Action as uncertain. (Code Civ. Proc., § 430.10, subd. (f).) "A demurrer for uncertainty is strictly construed, even where a complaint is in some respects uncertain, because ambiguities can be clarified under modern discovery procedures." (*Khoury v. Maly's of California, Inc.* (1993) 14 Cal.App.4th 612, 616.)

Here, the Medical Group calls the ground secondary and states that it “understands what Plaintiffs wish to prove,” adding that understanding a theory is not the same as being confronted with facts sufficient to constitute a cause of action. (Reply, at p. 6:11-13.) The qualification preserves the objection under Code of Civil Procedure section 430.10, subdivision (e), which the Court addressed above. It does not touch subdivision (f), which asks only whether the pleading is so unintelligible that a defendant cannot tell what it must admit or deny. The Demurrer for uncertainty is overruled.

CONCLUSION

The Motion to Quash Service of Summons is denied. Medical Group's Demurrer is overruled. Dr. Nelson's Demurrer is overruled. Medical

Group and Dr. Nelson shall file and serve an Answer within 20 days.